

Name:	ZAHMOUL, NADIA	Admit Date:	10/25/2025
MRN:	2018366	Discharge Date:	10/25/2025
Encounter:	5001134257	Attending:	DuRoss, John PA-C
DOB / Age:	4/27/1969	Copy to:	CareAware Oauth, SJMCWY
Sex / Birth Sex:	Female		

Office Clinic Notes

Document Type: Office Clinic Note Physician
Service Date/Time: 10/25/2025 17:06 MDT
Result Status: Auth (Verified)

ZAHMOUL, NADIA

DOB: 04/27/1969
Age: 56 years
Sex: Female
MRN: 2018366
Registration Date: 10/25/2025
Primary Care Physician:
No PCP, Physician

Chief Complaint

4 days sinus pressure, dryness, tired, swollen gland, sjogren's syndrome flair

History of Present Illness

56-year-old female with confirmed Sjogren syndrome by biopsy presents with concern for flare. She has been seeing her primary care provider here, Dr. Hayse, recently, but did not have flareup of symptoms 4 days ago when she last saw him. For sleep she took diphenhydramine, and feels like this made her significantly dry, and has had mouth and eye irritation, swollen glands and some sinus congestion. Has used prednisone for this in the past, last corticosteroid was over 6 months ago, taken for sinusitis. She also ran out of her hydroxychloroquine, 200 mg daily. Has asked Dr. Hayse office for a refill, but has not heard back and she took her last pill this morning.

Physical Exam

Vitals & Measurements

T: 36.9 °C (Tympanic) HR: 90 (Peripheral) RR: 18 BP: 119/83 SpO2: 96%
HT: 157 cm WT: 54 kg BMI: 21.91 BSA: 1.53

General: No acute distress, nontoxic

Eye: PERRL, EOMI, normal conjunctiva, no scleral injection

HENT: Non-erythematous ear canal bilaterally, bilateral tympanic membranes pearly gray w/ good cone of light, normal gross hearing, nares patent bilaterally, no focal sinus tenderness, mildly dry oral mucosa, normal dentition, oropharynx clear without tonsillar erythema/exudates.

Neck: Supple, non-tender, no JVD, no lymphadenopathy

Lungs: nonlabored, CTAB, no wheezing, rhonchi, or crackles

Heart: regular rate, regular rhythm, no murmur or rub.

Breast/ GU: defer

Abdomen: defer

Musculoskeletal: MAE, no peripheral edema

Skin: pink, warm, and dry

Neurologic: Awake, alert and oriented X4, no obvious focal deficits.

Problem List/Past Medical History

Ongoing

Anemia

Chronic rhinitis

Facial pressure

Nasal septal perforation

Sjogren syndrome

Historical

Chronic sinusitis

Facial pain syndrome

Nasal septal spur

Procedure/Surgical History

- Sinus X 2

Medications

New

hydroxychloroquine

(hydroxychloroquine 200 mg oral tablet) 1 tab Oral (given by mouth) every day for 30 Days. with food or milk.

Refills: 0.

prednisONE (prednisONE 20 mg oral tablet) 1 tab Oral (given by mouth) every day for 5 Days. Refills: 0.

Unchanged

dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral capsule, extended release) 1 Capsules Oral (given by mouth) every morning. Refills: 0.

St. John's Urgent Care

Name: ZAHMOUL, NADIA
MRN: 2018366
Encounter: 5001134257

Admit Date: 10/25/2025
Discharge Date: 10/25/2025
Attending: DuRoss, John PA-C

Office Clinic Notes

Depression Screen

PHQ 2

Little Interest - Pleasure in Activities: Not at all
Feeling Down, Depressed, Hopeless: Not at all
Initial Depression Screen Score: 0 Score

PHQ 9

Thoughts Better Off Dead or Hurting Self: Not at all

Assessment/Plan

Sjogren syndrome | (M35.00)

Orders (Last 48 Hours):

- hydroxychloroquine**(hydroxychloroquine 200 mg oral tablet), **200 mg= 1 tab, Oral, Daily**, with food or milk, (Ordered)
- predniSONE**(predniSONE 20 mg oral tablet), **20 mg= 1 tab, Oral, Daily**, (Ordered)
- 99213 Office Visit Level 3 Est**, 10/25/25 17:39:00 MDT, Sjogren syndrome, (Completed)

56-year-old female presenting this evening with concern for Sjogren's flare with increased oral mucosal dryness, eye irritation and sinus pressure. Per H&P, this is after taking diphenhydramine for sleep which has increased her overall dryness. Has run out of her hydroxychloroquine today, has been awaiting refill from PCP Dr. Hayse but not heard back. Using shared decision making, will refill her hydroxychloroquine as well as initiate prednisone burst therapy. Has tolerated 20 mg daily in the past with good results and low side effects. Prescribed for 5-day course, but she can stop after 2 to 3 days if symptoms improve. Otherwise if progression she should seek reevaluation. Can follow-up with PCP early next week. Focus on rest, hydration and avoid diphenhydramine.

Follow Up Instructions

With	When	Contact Information
Follow up with Primary Care Provider		
Additional Instructions: As discussed, I refilled your hydroxychloroquine. Take daily. Also for Sjogren's flare, take prednisone 20 mg daily for up to 5 days. No need to taper at this dose. If symptoms completely better after 2-3 days, can stop.		
Avoid Benadryl/diphenhydramine. Focus on gentle hydration, rest and seek reevaluation if symptoms are worsening. Otherwise, follow-up with primary care provider next week.		

dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral tablet)1 tab Oral (given by mouth) 2 times a day. Refills: 0.
estradiol (estradiol 0.025 mg/24 hours twice weekly transdermal film, extended release)Topical (on the skin) Mon/Tue/Thu/Fri/Sat.
FLUoxetine (FLUoxetine 20 mg oral capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.
FLUoxetine (FLUoxetine 20 mg oral capsule)
lisdexamfetamine (Vyvanse 70 mg oral capsule)1 Capsules Oral (given by mouth) every morning.
omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.
omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.
progesterone (progesterone 100 mg vaginal insert)
propranolol (propranolol 40 mg oral tablet)
sucralfate (Carafate 1 g oral tablet)1 tab Oral (given by mouth) 2 times a day. Refills: 0.

Allergies

No Known Allergies

Social History

Alcohol

Use:Never

Electronic Cigarette/Vaping

Electronic Cigarette Use:Never

Substance Use

Use:Never

Tobacco

Smoking tobacco use:Never tobacco user

Family History

Healthy adult: Mother and Father.

Electronically Signed on 10/25/2025 17:42 MDT

DuRoss, John PA-C